

PLEASE READ CAREFULLY	
KENO ORAL PASTE	
DESCRIPTION	the judgement of the physician or dentist the potential benefits outweigh the possible hazards.
A tasteless, odourless and viscous light-brown paste.	
COMPOSITION	SIDE EFFECTS
Each gram of Keno Oral Paste contains 1.0mg of Triamcinolone acetonide.	Prolonged administration may elicit the adverse reactions known to occur with systemic steroid preparations; for example, adrenal suppression, alteration of glucose metabolism, protein catabolism, peptic ulcer activations, and others. These are usually reversible and disappear when the hormone is discontinued.
INDICATIONS	Blurred vision may also be reported with systemic and topical corticosteroid use.
Keno Oral Paste is indicated for the adjunctive treatment and for the temporary relief of symptoms associated with inflammatory or ulcerative lesions of oral mucosa resulting from trauma.	
DOSAGE AND ADMINISTRATION	SYMPTOMS AND TREATMENT OF OVERDOSE
Squeeze a small amount of Keno Oral Paste onto a clean wet cotton bud and apply gently on to the affected area, without rubbing until a thin film develops. Apply at bed time and if necessary, 2 or 3 times daily after meals, or apply as directed by a doctor.	When used as recommended, systemic absorption is minimal and systemic effect is unlikely. Topical reactions to the corticosteroid have not been observed with triamcinolone acetonide when applied to the mouth.
CONTRAINDICATIONS	PHARMACODYNAMICS
This preparation is contraindicated in patients with a history of hypersensitivity to any of its components. Because it contains a corticosteroid, the preparation is contraindicated in the presence of fungal, viral or bacterial infections of the mouth or throat. It should not be used in herpetic lesions of known viral origin such as Herpes labialis or intraoral lesions such as primary herpetic gingivostomatitis gingivostomatitis and herpanginae.	Keno Oral Paste (triamcinolone acetonide) is a synthetic corticosteroid which possess anti-inflammatory, antipruritic, and anti-allergic action which may provide prompt relief of oral tenderness, pain, inflammation and ulceration. The emollient dental paste is an adhesive vehicle for applying medication to the oral surfaces. This adhesive maintains the medication in close contact with the lesion and provides a protective covering which augments the effects of the steroid.
PRECAUTIONS / WARNING	PHARMACOKINETICS
Patients with tuberculosis, peptic ulcer or diabetes mellitus should not be treated with any corticosteroid preparation without the advice of the patient's physician. It should be borne in mind that normal defensive responses of the oral tissues are depressed in patients receiving topical corticosteroid therapy. Virulent strains of oral micro-organisms may multiply without producing the usual warning symptoms of oral infections.	The absorption and fate of Triamcinolone is similar to corticosteroids. When administered by topical application particularly under an occlusive dressing or when the skin is broken, or as a pulmonary aerosol inhalation or a rectal enema, sufficient corticosteroid may be absorbed to give systemic effects. Corticosteroids in the circulation are extensively bound to plasma proteins, mainly to globulin and less so to albumin. The corticosteroid-binding globulin has high affinity but low binding capacity, while the albumin has low affinity but large binding capacity. Only unbound corticosteroid has pharmacological effects or is metabolised. Corticosteroids are metabolised mainly in the liver but also in the kidney, and are excreted in the urine. Triamcinolone is reported to have a biological half-life in plasma of about 300 minutes. It is bound to plasma albumin to a much smaller extent than hydrocortisone.
The small amount of steroid released when the preparation is used as recommended makes systemic effects very unlikely; however, there are a possibility when topical corticosteroid preparations are used over a long period of time and any unusual symptoms such as weakness or dizziness should be called to the physician's attention by the patient.	PRESENTATIONS
If local irritation or sensitization should develop, the preparation should be discontinued and appropriate therapy instituted.	Tube of 3g and 5g Box of 30 tubes x 1g (for single application only)
If significant regeneration or repair of oral tissues has not occurred in seven days, additional investigation into the aetiology of the oral lesion is advised.	STORE IN A COOL DRY PLACE, BELOW 30°C KEEP OUT OF THE REACH OF CHILDREN JAUHI DARIPADA KANAK-KANAK SHELF LIFE: Please refer to outer box label
Visual disturbances may be reported with systemic and topical corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes which may include cataract, glaucoma or rare diseases such as central serous chorioretinopathy (CSCR) which have been reported after use of systemic and topical corticosteroids.	For application in the oral cavity.
Phaeochromocytoma crisis, which may be fatal, has been reported after administration of systemic corticosteroids. Corticosteroids should only be administered to patients with suspected or identified phaeochromocytoma after an appropriate risk/benefit evaluation.	MANUFACTURED BY: T.O. Chemicals (1979) Ltd., 280 Soi Sabalai, Suthisarnwinjai Road, Samsenok, Huay-Kwang, Bangkok 10310, THAILAND
DRUG INTERACTIONS	Last reviewed: July 2022
There is no known interaction.	
PREGNANCY AND LACTATION	
Safe use of this preparation during pregnancy has not been established with respect to possible adverse reactions upon foetal development; therefore, it should not be used in women of child-bearing potential and particularly during early pregnancy unless in	

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